Patient Information

Name: Henry KING Zee
DOB: 01/09/1948
Address: 7340 Red Head Dr
North Las Vegas NV 89084
Phone: 818-436-9916
818-436-9916

Primary Payor: Medicare
ID: 7QV8PH3PE30
Grp. No. N/A
Claim Address: 900 42ND STREET SOUTH
FARGO, ND 58103-6776

Secondary Payor: AARP/AARP
ID: 08629561711
Claim Address: UNITEDHEALTHCARE CLAIMS DIVISION
PO BOX 1878
SOUTHAMPTON, PA 18966-9998

Referred TO

Provider
Provider:
Specialty: Home Health Services
Address: No referred to provider on referral.
Phone: 702-416-0611
Fax: None

Location
Location Dept.: Avery Home Health
Specialty: Home Health Services
Address: 5660 W Flamingo Rd Ste. A
LAS VEGAS NV 89103
Phone: 702-416-0611
Fax: 702-899-4479

Referred BY

Provider: Aditi Singh **Attending Provider**: No info available
Location: UNLV Internal Medicine C* **Dept.:** UNLV Internal Medicine - General Clinic
Address: 1707 W. Charleston Blvd. #230
LAS VEGAS NV 89102-2353
Phone: 702-671-5060 **Fax:** 702-671-5198

Referral Information

Authorization #
Authorization Comments:
Start Date: 02/05/2026
End Date: 02/05/2027
Visits: 99

Diagnoses:
R41.89 (ICD-10-CM) - Cognitive impairment
R26.89 (ICD-10-CM) - Balance problem

Procedures:
REF34 - AMB REFERRAL TO HOME HEALTH

Appointment Information

Date:
Time:
Location:

Electronically Signed by: SINGH, ADITI Jan 22, 2026 5:16 PM

UNLV Internal Medicine - General Clinic
1707 W. Charleston Blvd. #230
LAS VEGAS NV 89102-2353
Phone: 702-671-5060
Fax: 702-384-6609

Patient:
Henry KING Zee
7340 Red Head DR
NORTH LAS VEGAS NV 89084
Phone: 818-436-9916

CSN: 100060323198
MRN: 1000264318
DOB: 1/9/1948
SSN: xxx-xx-5608
Sex: M

INSURANCE	PAYOR	PLAN	GROUP #	SUBSCRIBER ID
Primary:	MEDICARE	40000001		7QV8PH3PE30
Secondary:	AARP	50000101	PLAN G	08629561711

Order Date: Jan 22, 2026

Ambulatory Referral to Home Health (Order ID: 129792092)

Diagnosis: Cognitive impairment (R41.89)
Balance problem (R26.89)
Priority: Routine
Reason for exam:
Comments: Please evaluate Henry KING Zee for admission to Home Health.

Disciplines requested: Nursing, Physical Therapy, and Home Health Aide

Services to provide: Strengthening Exercises and Evaluate

Physician to follow patient's care (the person listed here will be responsible for signing ongoing orders): PCP

Requested Start of Care Date: Within 2-3 days

Special Instructions:
Patient with memory difficulty, could use assistance with medications, checking vitals, PT and strengthening

I attest that I or another qualified licensed provider saw Henry KING Zee 90 days prior to or 30 days post admission and this face to face encounter meets the necessary Home Health requirements. The face to face encounter occurred on (date)1/22/26

I certify that, based on my findings, the following services are medically necessary skilled home health services: Strengthening Exercises and Evaluate.

Location:

Ordered by: Aditi Singh, MD
Authorized by: Aditi Singh, MD (NPI: 1033234372)
Electronically signed by: SINGH, ADITI (NPI: 1033234372)

Zee, Henry KING

MRN: 1000264318

Office Visit 1/22/2026
UNLV Internal Medicine - General Clinic

Provider: Aditi Singh, MD (Internal Medicine)
Primary diagnosis: Cognitive impairment
Reason for Visit: Medicare Annual Wellness Visit Subsequent

Progress Notes

Aditi Singh, MD (Physician) • Internal Medicine

UNLV HEALTH

UNLV INTERNAL MEDICINE - GENERAL CLINIC
1707 W. CHARLESTON BLVD. #230
LAS VEGAS NV 89102-2353
702-671-5060

Subjective
Henry KING Zee is a 78 y.o. male
Chief Complaint: Medicare Annual Wellness Visit Subsequent

HPI
Mr. Zee is a 78 year old male patient here to establish care with me

He is accompanied by his daughter, and her son is on phone as well

CAD
Follows with Dr. Diep - has had CABG x 2
He is on ASA, statin
Denies CP, palpitations

Hypertension- the patient has been compliant with blood pressure medications, denies any side effects of the medications. The patient denies chest pain, shortness of breath, or palpitiation. Elevated today, but at home it is usually 125-130 systolic

Memory impairment
His minicog is 0
Word recall and recognition difficulty
He had investigation in May 2025, but since August hospitalization it has acutely worsened
His daughter and son voice concerns as he has left stove on as well
I reviewed prior records with neurology, MRA neck and head reviewed
He was referred to balance therapy

Atrial fibrillation
Rate controlled and on eliquis

Hypertension- the patient has been compliant with blood pressure medications, denies any side effects of the medications. The patient denies chest pain, shortness of breath, or palpitiation.

Last colonoscopy -- 2025 - hemorrhoids noted as well as polyps
Low Dose CT -- quit tob 40 years ago
Influenza -- done
PPSV23 -- done
PCV13 -- done
Tdap -- 2018
Shingrix -- Done
COVID vaccine -- Pfizer x3 -- per prior notes does not want additional boosters due to risks with booster
HCV -- neg 2021
Hep B -- neg, not immune 2024

AAA Screen -- done per prior notes
Urine microalb/cr - ordered
Eye exam - to be done
Advanced Directives -- discussed with patient and daughter; per patient his wife knows his wishes
Fall risk -- has been referred to PT in past, per son he had 2 visits then patient deferred, agreeable to home health
Visual acuity -- see note
Abnormal cognitive screen -- reviewed neurology input from last year, referred again

Annual Wellness Questionnaire
In general, how would you describe your health?: Good
Do you have pain that interferes with performing desired activities?: Yes
How often do you excercise?: Rarely
How would you describe your diet?: General Healthy
During the past 4 weeks, how many drinks of wine, beer or alcoholic beverages did you have?: 0
Do you use drugs? If so, how often?: Never
Are you a smoker? : No
Are you currently using opiods/narcotics?: No
Do you have difficulty with your hearing? : No
How often during the past 4 weeks have you been bothered by fall/dizzy when standing up?: Occasionally
How often during the past 4 weeks have you been bothered by your teeth or dentures?: Rarely
How often during the past 4 weeks have you been bothered by being tired or fatigued? : Most days
Does your home have rugs in the hallways? : Yes
Does your home have grab bars in the bathrooms?: Yes
Does your home have stairs?: No
How often do you have trouble taking your medications as prescribed? : Never
How confident are you that you can control and manage most of your health problems? : Confident

Problem List
Patient Active Problem List
Diagnosis
- Testicular cyst
- Essential hypertension
- Gout
- Mixed hyperlipidemia
- Chronic kidney disease, stage III (moderate) (CMS/HCC)
- Vitamin D deficiency
- Right inguinal pain
- Type 2 diabetes mellitus
- Preventative health care
- Coronary atherosclerosis of autologous vein bypass graft
- Colon polyp
- Peripheral vascular disease (CMS/HCC)
- History of 2019 novel coronavirus disease (COVID-19)
- Tinnitus of both ears
- Bilateral hearing loss
- Cervical disc disease
- Prediabetes
- A-fib (CMS/HCC)
- Spinal stenosis of lumbar region with neurogenic claudication
- Chronic midline low back pain without sciatica
- Obstructive sleep apnea syndrome
- History of fall
- Negative depression screening
- Former cigarette smoker
- Left shoulder pain
- Blister of right heel

- Skin lesion of hand
- Cognitive impairment
- Fatigue
- History of hemorrhoidectomy
- Gastrointestinal hemorrhage
- Chronic idiopathic constipation

Past Medical History:

Diagnosis	Date
• A-fib (CMS/HCC)	
• Bilateral hearing loss	01/21/2021
• BPPV (benign paroxysmal positional vertigo)	2005
• Cervical disc disease	07/29/2021
• Chronic kidney disease, stage III (moderate) (CMS/HCC) *Kidney stone*	06/25/2018
• Chronic midline low back pain without sciatica	09/01/2022
• Cognitive impairment	03/03/2022
• Colon polyp *last colonoscopy 2019*	
• Coronary atherosclerosis of autologous vein bypass graft *CABG 5V 1985; CABG 4V 2003 Cleveland Clinic (SVG to diag, radial to OM2, radial to PDA, RIMA to LAD); MI 2008; ACS 2012 occluded diag,RCA 90%; med managemenbt*	1985
• Essential hypertension	06/25/2018
• Family history of colon cancer	
• Gout *2 attacks per year*	06/25/2018
• Mixed hyperlipidemia	06/25/2018
• Nephrolithiasis	
• Obstructive sleep apnea syndrome	09/01/2022
• Peripheral vascular disease (CMS/HCC) *ABI R 1.04; L 1.57*	2015
• PPD positive	2001
• Rotator cuff tendonitis, left	
• Status post hemorrhoidectomy	08/31/2025
• Stercoral colitis	08/31/2025
• Testicular cyst *ultrasound 1.5cm testicular cyst, L 3mm epidydymal cyst, small B hydroceole; saw GU Weiss and no interventions advised.*	2017
• Type 2 diabetes mellitus	2016
• Vitamin D deficiency	

Past Surgical History:

Procedure	Laterality	Date
• APPENDECTOMY OPEN		
• CATARACT EXTRACTION		
• CORONARY ARTERY BYPASS GRAFT		
• INGUINAL HERNIA REPAIR	Right	2016

Social History

Social History

Tobacco Use

• Smoking status:	Former
Types:	Cigarettes
• Smokeless tobacco:	Never

Substance Use Topics

• Alcohol use:	Yes
Comment: rare	
• Drug use:	No

Allergies

No Known Allergies

Current Medications

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• apixaban (ELIQUIS) 2.5 mg tablet	Take 1 tablet (2.5 mg total) in the morning		

	and 1 tablet (2.5 mg total) before bedtime by mouth.		
• ascorbic acid (VITAMIN C) 500 mg/5 mL syrup	Take 2.5 mL (250 mg total) in the morning by mouth.		
• aspirin 81 mg EC tablet	Take 1 tablet (81 mg total) in the morning by mouth. Ld 08/7/19.		
• atorvastatin (LIPITOR) 40 mg tablet	Take 1 tablet (40 mg total) nightly by mouth	90 tablet	1
• carvediloL (COREG) 12.5 mg tablet	TAKE 1 TABLET BY MOUTH IN THE MORNING AND 1 TAB IN THE EVENING	180 tablet	1
• cholecalciferol, vitamin D3, 25 mcg (1,000 unit) capsule	Take 1 capsule (1,000 Units total) in the morning by mouth. 8/7/19.		
• losartan (COZAAR) 100 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY	90 tablet	1
• magnesium chloride 64 mg tablet,delayed release (DR/EC)	Take 535 mg in the morning by mouth.		
• melatonin 5 mg capsule	Take by mouth		
• nitroglycerin (NITROSTAT) 0.3 mg SL tablet	PLACE 1 TABLET UNDER TONGUE EVERY 5 MINS, UP TO 3 DOSES AS NEEDED FOR CHEST PAIN NO RELIEF CALL 911	100 tablet	0
• ranolazine (RANEXA) 500 mg 12 hr tablet	Take 1 tablet (500 mg total) in the morning and 1 tablet (500 mg total) before bedtime by mouth.		
• vitamin E 400 unit capsule	Take 1 capsule (400 Units total) in the morning by mouth. Ld 8/7/19.		
• hydroCHLOROthiazide (HYDRODIURIL) 12.5 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY	90 tablet	1

No current facility-administered medications for this visit.

Family History

Problem	Relation	Age of Onset
• Colon cancer	Mother	75
• Suicide Attempts *completed suicide at age 77yo*	Mother	77
• Coronary artery disease	Father	
• Stroke *died age 84yo*	Father	
• Diabetes	Brother	
• Heart attack	Maternal Grandfather	

Patient Care Team:
Aditi Singh, MD as PCP - General (Internal Medicine)

Review of Systems

Constitutional: Negative.
Respiratory: Negative.
Cardiovascular: Negative.

Vitals:

	01/22/26 1546	01/22/26 1645
BP:	**(!) 171/90**	**(!) 160/82**
BP Location:	Left arm	Left arm
Patient Position:	Sitting	Sitting
Pulse:	62	64
Resp:	18	
Temp:	36.1 °C (97 °F)	
TempSrc:	Temporal	
SpO2:	96%	
Weight:	73.5 kg (162 lb)	
Height:	167.6 cm (5' 6")	

Pain Assessment
Pain Score: 6
Body mass index is 26.15 kg/m².

Visual Acuity
Visual Acuity - Bilateral Eyes: 20/40
Visual Acuity - Right Eye: 20/50
Visual Acuity - Left Eye: 20/50
Vision - Corrective Lenses: None

Mini-Cog
Step 1 : Three Word Registration: Instruction Given
Step 2 : Clock Drawing: Incorrect
Step 3: 1st word: 0
2nd Word: 0
3rd Word: 0
Mini-cog Score: 0

Depression/Anxiety Screen
Little Interest or Pleasure in Doing Things: Not at all
Feeling Down, Depressed, or Hopeless: Not at all
PHQ-9 Total Score: 0
Interpretation:Negative

Functional Ability and Safety
Self-care Ability: Independent with self-care.
Family and Social Ability: Able to perform all.
Movement Ability: Able to get up and walk independently, able to climb one flight of stairs.
Lifting Ability: Able to lift up to 20 lbs.
Work Ability: Unable to do any work.
Functional Ability Score: 65

Fall Risk Assessment
Are you afraid of falling?: **(!) Yes**

Advanced Care Planning
has NO advanced directive - add't info requested.

Written Screen Schedule
Immunization History

Administered	Date(s) Administered
• Hepatitis A	10/09/2018, 09/24/2019
• Influenza (IM) Preservative Free	10/12/2011
• Influenza Split High Dose Preservative Free IM	10/24/2017, 09/21/2018, 09/06/2024, 09/30/2025
• Influenza, High-Dose, Quadrivalent	09/24/2020, 10/01/2021, 09/23/2022
• Influenza, Injectable, Quadrivalent, Preservative Free Vaccine	09/12/2019
• Influenza, Quadrivalent	08/28/2023
• Influenza, Unspecified	11/28/2006, 11/01/2008, 09/24/2020, 10/01/2021, 10/20/2022, 09/10/2024
• Pfizer Sars-cov-2 Vaccination (EUA)	01/29/2021, 02/19/2021, 10/19/2021
• Pneumococcal Conjugate 13-Valent	09/16/2016

• Pneumococcal Polysaccharide	04/08/2013, 09/24/2019
• Pneumococcal, Unspecified	10/20/2022
• Rsv, Recombinant, Protein Subunit Rsvpref	09/30/2025
• TD Preservative Free	01/11/2005
• Tdap	07/01/2022
• Typhoid Inactivated	10/09/2018
• zoster recombinant	09/24/2019, 11/22/2019

Health Maintenance

Topic	Date Due
• Diabetes Screening: A1c Monitoring Every 1 Year	01/13/2026
• Vaccine: DTaP,Tdap,and Td (2 - Td or Tdap)	07/01/2032
• Vaccine: Influenza	Completed
• Vaccine: Pneumococcal (65+ Years)	Completed
• Colorectal Cancer Screening	Discontinued

Have you received your entire Covid-19 Vaccine Series: Yes - Documented in Chart (1/22/2026 3:47 PM)
Have you received your Influenza Vaccine for this Flu season?: Yes - Documented in chart. (1/22/2026 3:47 PM)
Have you received your Pneumococcal Vaccine?: Yes - Documented in Chart. (1/22/2026 3:47 PM)

Physical Exam
Vitals and nursing note reviewed.
Constitutional:
Appearance: He is well-developed.
HENT:
Head: Normocephalic and atraumatic.
Eyes:
Extraocular Movements: Extraocular movements intact.
Conjunctiva/sclera: Conjunctivae normal.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Heart sounds: Normal heart sounds.
Pulmonary:
Effort: Pulmonary effort is normal.
Breath sounds: Normal breath sounds.
Neurological:
Mental Status: He is alert and oriented to person, place, and time.
Psychiatric:
Behavior: Behavior normal.

Problem List Items Addressed This Visit

Essential hypertension
Elevated today
Per patient and his family, much improved at home. Today it was 126/80

Relevant Orders
Comprehensive metabolic panel
Microalbumin, Urine, Random

Mixed hyperlipidemia
Preventative health care
Last colonoscopy -- 2025 - hemorrhoids noted as well as polyps per family
Low Dose CT -- quit tob 40 years ago
Influenza -- UTD
PPSV23 -- UTD
PCV13 -- UTD
Tdap -- 2018

Shingrix -- Done
COVID vaccine -- Pfizer x3 -- per prior notes does not want additional boosters due to risks with booster
HCV -- neg 2021
Hep B -- neg, not immune 2024
AAA Screen -- done per prior notes
Urine microalb/cr - ordered
Eye exam - to be done
Advanced Directives -- discussed with patient and daughter; per patient his wife knows his wishes
Fall risk -- has been referred to PT in past, per son he had 2 visits then patient deferred, agreeable to home health
Visual acuity -- see note
Abnormal cognitive screen -- reviewed neurology input from last year, referred again

Coronary atherosclerosis of autologous vein bypass graft
Relevant Orders
Hemoglobin A1c
Lipid panel

A-fib (CMS/HCC)
Relevant Orders
TSH with Reflex to Free T4

Negative depression screening
Cognitive impairment - Primary
Discussed patients decline in cognition over time
Since August when he was hospitalized, his children mention it has considerably worsened.
Additionally they mention changes in behavior including change in activities and appetite, increased sleep
Discussed with patient and he denies any symptoms of depression or SI at this time but we will continue to discuss
I referred for neurology, order labs and ordered MRI. I did review Steinberg imaging, MRI has not been done recently.

Relevant Orders
MR Brain with and without contrast
TSH with Reflex to Free T4
Urinalysis with Reflex to Microscopic
Ambulatory referral to Neurology
Ambulatory Referral to Home Health

Other Visit Diagnoses

Hemorrhoids, unspecified hemorrhoid type
Relevant Orders
CBC with auto differential

Balance problem
Relevant Orders
Ambulatory Referral to Home Health

Assessment & Plan Aditi Singh, MD

Other Notes

All notes

- Preventative health care A&P from Aditi Singh, MD (Internal Medicine)
- Essential hypertension A&P from Aditi Singh, MD (Internal Medicine)
- Cognitive impairment A&P from Aditi Singh, MD (Internal Medicine)

Instructions

Return in about 3 months (around 4/22/2026).

After Visit Summary - Printed 1/22/2026

Additional Documentation

Vitals: BP 160/82 ! (Abnormal) (BP Location: Left arm, Patient Position: Sitting) Pulse 64
Temp 36.1 °C (97 °F) (Temporal) Resp 18 Ht 167.6 cm (5' 6") Wt 73.5 kg (162 lb) SpO2 96%
BMI 26.15 kg/m² BSA 1.85 m² Pain Sc 6 (Loc: Leg)

Flowsheets: Fall Risk Screening, PHQ-9, Vaccine Assessment, Vital Signs, Vitals Reassessment, APACHE IV, Functional Ability, Annual Wellness Questionnaire, Visual Acuity, Mini Cog

Communications

Orders Placed

CBC with auto differential
Comprehensive metabolic panel
Hemoglobin A1c
Lipid panel
Microalbumin, Urine, Random
TSH with Reflex to Free T4
Urinalysis with Reflex to Microscopic
MR Brain with and without contrast
Ambulatory referral to Neurology Pending Review
Ambulatory Referral to Home Health Pending Review

Medication Changes

As of 1/22/2026 3:54 PM

None

Medication List at End of Visit

As of 1/22/2026 3:54 PM

	Refills	Start Date	End Date
apixaban (ELIQUIS) 2.5 mg tablet	—		—
Take 1 tablet (2.5 mg total) in the morning and 1 tablet (2.5 mg total) before bedtime by mouth. - oral			
Patient-reported medication			

	Refills	Start Date	End Date
ascorbic acid (VITAMIN C) 500 mg/5 mL syrup Take 2.5 mL (250 mg total) in the morning by mouth. - oral Patient-reported medication	—		—
aspirin 81 mg EC tablet Take 1 tablet (81 mg total) in the morning by mouth. Ld 08/7/19. - oral Patient-reported medication	—		—
atorvastatin (LIPITOR) 40 mg tablet Take 1 tablet (40 mg total) nightly by mouth - oral	1	1/15/2025	—
carvediloL (COREG) 12.5 mg tablet TAKE 1 TABLET BY MOUTH IN THE MORNING AND 1 TAB IN THE EVENING	1	1/16/2025	—
cholecalciferol, vitamin D3, 25 mcg (1,000 unit) capsule Take 1 capsule (1,000 Units total) in the morning by mouth. 8/7/19. - oral Patient-reported medication	—		—
hydroCHLOROthiazide (HYDRODIURIL) 12.5 mg tablet TAKE 1 TABLET BY MOUTH EVERY DAY	1	1/15/2025	—
losartan (COZAAR) 100 mg tablet TAKE 1 TABLET BY MOUTH EVERY DAY	1	1/15/2025	—
magnesium chloride 64 mg tablet,delayed release (DR/EC) Take 535 mg in the morning by mouth. - oral Patient-reported medication	—		—
melatonin 5 mg capsule Take by mouth - oral Patient-reported medication	—		—
nitroglycerin (NITROSTAT) 0.3 mg SL tablet PLACE 1 TABLET UNDER TONGUE EVERY 5 MINS, UP TO 3 DOSES AS NEEDED FOR CHEST PAIN NO RELIEF CALL 911	0	4/24/2021	—
ranolazine (RANEXA) 500 mg 12 hr tablet Take 1 tablet (500 mg total) in the morning and 1 tablet (500 mg total) before bedtime by mouth. - oral Patient-reported medication	—		—
vitamin E 400 unit capsule Take 1 capsule (400 Units total) in the morning by mouth. Ld 8/7/19. - oral Patient-reported medication	—		—

Visit Diagnoses

Primary: **Cognitive impairment** R41.89
Atherosclerosis of autologous vein coronary artery bypass graft, unspecified whether angina present I25.810
Essential hypertension I10
Paroxysmal atrial fibrillation (CMS/HCC) I48.0
Hemorrhoids, unspecified hemorrhoid type K64.9
Balance problem R26.89
Mixed hyperlipidemia E78.2
Negative depression screening Z13.31
Preventative health care Z00.00